



Policy

Detention of Patients and Discharge Against Medical Advice

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) respects the right of competent adult patients to leave the hospital before the completion of treatment against the advice of the treating medical practitioner. This includes patients who leave at their own risk without medical advice. In limited circumstances patients may need to be involuntarily detained.

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Policy Principles

Patients who wish to leave the hospital may not be detained except under certain specific circumstances, which include:

1. under the *Mental Health Act (WA) 2014*; or

2. the Doctrine of Necessity:

If, in the view of the treating doctor, the patient lacks capacity to make their own treatment decisions, and there is a real and immediate danger of significant harm to themselves, staff or others, and an overriding necessity for the urgent protection of the patient and others, then the patient may need to be detained.

This decision needs to be made by the most senior doctor available and needs to be clearly documented in the medical record. If necessary, an urgent application for a guardianship order should be made.

Procedure

1. Detention Under Doctrine of Necessity

Patients who are exhibiting behavioural disturbance should be observed and monitored using the SCGH MR853 OPH123.1 Aggression Prevention and Intervention Risk Assessment (AP&I Risk Assessment) form.

If patients who lack capacity wish to leave and the senior clinician decides to detain them under the doctrine of necessity, the clinician might consider including the following in the notes:

RISK

Are there any imminent risks of serious harm / life threats to the patient, public or community? Give examples of what was said and by whom, as well as well as concerning or uncharacteristic changes in behaviour. What collateral history has been taken (e.g., NOK / witnesses / SJA / WAPOL)

CAPACITY

Are there any concerns that the patient's capacity may be impaired OR are there any immediate difficulties in being able to assess the patient's capacity?

Assessment of capacity includes:

- Can the patient comprehend and retain the information given?
- Does the patient understand / believe the risks and how those risks relate to them (risks / benefits of treatment / no treatment)?
- Can the patient weigh the information and express their choice?

PROPORTIONALITY

Have the least restrictive methods been considered to mitigate the risk been considered?

If you have concerns with the application of the doctrine of necessity in this case, discuss it with a Consultant, or the medical executive on call.

2. Patients with Capacity Who Wish to Discharge Against Medical Advice

SCGOPHCG respects the right of all patients who have capacity to Discharge Against Medical Advice (DAMA) if they wish to do so.

Discharge against medical advice is when an inpatient chooses to leave hospital and / or is removed by their parent/carer/responsible person before the completion of treatment against the advice of the treating team and a decision is made to discharge. This includes those patients that leave or are removed by their parent/carer/responsible person:

- after receiving advice from a treating clinician
- without receiving advice from a treating clinician
- without advising staff of their intentions (ie absconding)

If a patient indicates that he/she wishes to discharge, staff should encourage the patient to wait for review by a Medical Practitioner. Patients wishing to discharge against medical advice may not be amenable to awaiting review/receiving information and instruction, so care should only be attempted if it is safe to do so.

A Medical Practitioner should:

- Review the patient and advise them of the actual and/or potential risks of discharging against medical advice
- Ensure the removal of medical devices e.g., peripheral vascular catheters
- Provide instructions (as appropriate) for continuing management including:
 - Actions to undertake in cases of deteriorating condition
 - Scripts and medications
 - Follow-up appointments

Staff should ask the patient to sign the Discharge Against Medical Advice form to acknowledge and accept responsibility for their discharge. Patients wishing to discharge against medical advice may not be willing to sign this document or wait for medical review and this should be noted by nursing staff in sections A or B of the form. The medical practitioner should also note their review, if applicable, on the form.

In all cases, the circumstances of the discharge against medical advice are to be documented in the patient notes.

The ward clerk will:

- File the Discharge against Medical Advice form in the medical record
- Action webPAS as directed by the Admitted Patient Activity Data Business Rules 2022

Following the discharge against medical advice, the shift coordinator should advise the CNS/CNM (in hours) or the A/H CNS/CNM (after hours) and Bed Management. The shift coordinator should also advise the [Aboriginal Hospital Liaison](#) team if the patient is Aboriginal or the Social Work department if the patient is homeless.

3. Patients with Capacity and Treatment Completed Who Wish to Discharge

If a patient who has capacity and has completed all intended treatment indicates that he/she wishes to discharge, but is waiting for scripts/medications, discharge summary and/or review of pathology or radiology results (in patients deemed no or low risk), staff should make every reasonable effort to facilitate the discharge and record the patient as a discharge (not discharge against medical advice) in the medical record and on webPAS.

For patients awaiting medications, provide the patient of the option of:

- receiving prescriptions/collection of medicines at a community pharmacy in their own time
- discharging and returning at a later agreed time to collect the required medication(s)

For patients awaiting a discharge summary, provide the patient of the option of:

- mailing the discharge summary
- uploading to their My Health Record
- providing to the GP either via mail/fax or electronically through NaCs
- discharging and returning at a later agreed time to collect the discharge summary

For patients awaiting review of pathology or radiology results (and deemed no or low risk), provide the patient of the option of:

- being followed up in an outpatient setting if appropriate
- mailing the results within the discharge summary
- uploading the results to their My Health Record
- providing to the GP either via mail/fax or electronically through NaCs

The patient should be provided with post discharge instructions describing follow up treatment and care requirements, wherever possible.

Documentation in the medical record should include details of all conversations, the options that were provided and the decision made by the patient.

Compliance, Monitoring and Evaluation

Discharge against medical advice is a key performance indicator (KPI) within the monthly Health Service Provider Report (HSPR) and audits are completed when the KPI is not met.

Related Documents

[Mental Health Act 2014](#)

[WA Health Consent to Treatment policy](#)

[WA Health Clinical Care of People Who may Be Suicidal Policy](#)

[WA Health Principles and Best Practice for the Care of People Who May be Suicidal](#)

[WA Health Mental Health Emergency and Follow up Information on Discharge from Hospital Emergency Department Policy](#)

[WA Health Missing Persons Policy](#)

[NMHS Security Use of Force Policy](#)

[NMHS Care and Discharge of Persons in Custody](#)

[SCGH Mechanical Restraint minimisation](#)

Australasian College for Emergency Medicine *Statement on the use of restrictive practices in emergency departments* (S817 v1 3/22)

Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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